

The American Health Ownership Plan (AHOP)

Universal coverage, no medical bankruptcy, no job lock — built to survive a divided Congress

The premise. Health care is the largest source of financial ruin in American life, and coverage is chained to a job you can lose tomorrow. AHOP delivers universal coverage with guaranteed issue — nobody uninsurable, nobody bankrupted, nobody trapped in a bad job for the insurance. It reaches those goals through a private-insurer architecture rather than a public payer, which is precisely why it can pass: it is designed to win the votes that single-payer proposals cannot, without surrendering the outcomes that make them worth wanting.

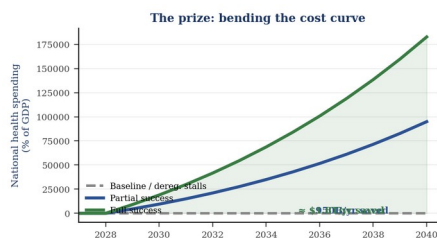
The architecture.

LAYER 1 Coverage for everyone	Universal coverage — every resident insured, with guaranteed issue , community-style rating, and a risk-adjustment formula written into statute so insurers cannot cherry-pick the healthy. Deductibles and out-of-pocket exposure scale with income and are capped: no family is bankrupted by illness. A smart deductible puts high-value care outside cost sharing entirely — preventive screenings, prenatal care, mental-health maintenance, insulin, inhalers, and blood-pressure medication are covered from the first dollar.
LAYER 2 Accounts working families own	Health savings accounts for routine care, seeded with direct public deposits for lower-income households — progressive transfers replacing today's opaque subsidy machinery. The account is the family's property forever. Comparison tools and binding quotes are required by design: the burden falls on software, not on sick people. No one is ever asked to shop for cancer or emergency care — that sits above the deductible, where insurers negotiate on the patient's behalf.
LAYER 3 A safety net that cannot be starved	A guaranteed backstop for the poor and chronically ill, in a dedicated trust with dedicated revenue and a statutory adequacy floor that automatically adjusts revenue if funding would fall short. Unlike an appropriation, it cannot be quietly squeezed in a budget fight — it can only be cut by a recorded vote against the sick and the poor. Funded in part by a Solidarity Share on high earners' above-cap contributions.
FOUNDATION Breaking up the hospital cartel	Confronts consolidated provider markets directly: repeal of anti-competitive entry barriers, expanded clinician supply, binding price transparency, site-neutral payments that end the hospital-ownership billing premium, and faster generics and biosimilars. Backed by a hard guarantee: where markets are concentrated and prices fail to fall, reference pricing capped at a multiple of Medicare rates activates automatically — and providers must accept it to keep federal program participation.

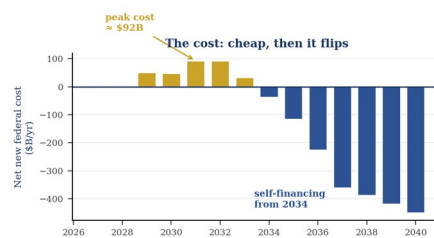
What it does for people. Lose your job and nothing lapses — the policy and the account are yours, and because subsidies scale with income, a layoff automatically means a lower deductible and larger support. COBRA becomes obsolete, and job lock ends. Chronic-disease care — insulin, inhalers, mental-health maintenance — is free at the point of use, because a skipped \$40 refill becomes a \$20,000 emergency the system pays for anyway.

Who pays. Not working families. Financing comes from ending the regressive employer tax exclusion — a \$330B/yr break that disproportionately benefits high earners with gold-plated plans — with a statutory paycheck conversion legally requiring employers to hand those dollars back as wages. Existing Medicaid, CHIP, and ACA streams migrate in; high earners contribute through the Solidarity Share. The median household is provably no worse off.

The numbers (14-year model, 12-run sensitivity grid). Universal coverage at a peak cost of roughly \$92B/yr — a fraction of one year of Medicare — and the ledger flips to savings by 2034. Every sensitivity variation stays favorable: the parameters move the price and timing, not the sign. And the system-wide prize is contingent on confronting provider consolidation, which is exactly where the left has been right all along.



Taking on provider consolidation is where the savings live.



Universal coverage for ≈ \$92B/yr at peak — then it pays for itself.

Why this one can pass. Every repeal-era bill scored as millions losing coverage; this one scores as a coverage gain — the weapon that killed 2017 does not exist here. It is severable by design: hospital-consolidation and price-transparency reform can move now, and covering every child in America (Title II) revives the CHIP precedent, the most passable bill in health policy. The rest is trigger-gated for the Medicare trust-fund window of the early 2030s. It is not a bill to introduce — it is the architecture to have ready when the window opens.